Access guide — aml-mds-related-rr-tp53-aberrant-hla-pending-x7q2-rerun2

How to access each therapy: trial contacts + manufacturer medical-info

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Access guide — aml-mds-related-rr-tp53-aberrant-hla-pending-x7

How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a Verified date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Donor lymphocyte infusion (DLI) from the HLA-identical sibling donor	other	Standard of care + Clinical trial	Established post-transplant practice; cells are collected and infused under the transplant programme's own authorisations rather than as a licensed product.	Ask the original transplant centre what donor cells are still available and whether the sibling would consent to a fresh collection, since that answer takes time to obtain
2	Extramedullary myeloid sarcoma (right thigh) — assessment and local control	other	Standard of care	Not a drug. Biopsy, cross-sectional imaging and local radiotherapy are established clinical services rather than regulated products.	Ask whether the thigh lesion can be biopsied for site-specific immunophenotype and genotype rather than assuming the marrow result applies to it
3	Fractionated gemtuzumab ozogamicin (CD33-directed antibody-drug conjugate)	ADC	Standard of care + Clinical trial	FDA-approved (MYLOTARG). The label carries two indications: newly-diagnosed CD33-positive AML in adults and paediatric patients 1 month and older, and relapsed or refractory CD33-positive AML in adults and paediatric patients 2 years and older. EMA-approved for newly-diagnosed CD33-positive AML.	Order quantitative CD33 antigen-density flow before committing, since it is the variable that separates a reasonable ADC bet from a futile one

4	Pending diagnostics that gate the ranked options (HLA class I typing, HA-1/HA-2 genotyping, quantitative CD33 and CD123, TP53 variant-level NGS with CNV, HLA-LOH on blasts)	other	Standard of care	Laboratory testing rather than a regulated therapeutic. High-resolution HLA typing runs in ASHI-accredited histocompatibility laboratories; TP53 sequencing, p53 17p FISH and quantitative flow run in CLIA-certified clinical laboratories.	Request the 2020 high-resolution HLA typing reports for the patient and the sibling donor from the transplant centre or donor registry before ordering any new test; retrieval is faster than repeat typing and A*02:01 gates two of the best options
5	Second allogeneic haematopoietic cell transplant (HCT2)	other	Standard of care + Clinical trial	Established procedure rather than a regulated product; conditioning agents are individually approved, and treosulfan was FDA-approved in the US in 2025 for use in allogeneic transplant conditioning.	Get a transplant-programme consultation booked now rather than after salvage, since donor work-up and authorisation run on a longer clock than chemotherapy
6	CLAG-GO (cladribine, high-dose cytarabine, G-CSF plus fractionated gemtuzumab ozogamicin)	other	Off-label use + Clinical trial	Regimen of individually approved agents. Gemtuzumab ozogamicin is FDA-approved for relapsed/refractory CD33-positive AML; cladribine, cytarabine and filgrastim are approved products used off-label in this combination.	Call the University of Maryland study contacts (410-328-9416 / veronica.kflu@umm.edu) this week and ask about slot availability and screening turnaround for NCT04050280
7	FLAMSA-RIC (sequential with reduced-intensity conditioning and prophylactic DLI)	other	Off-label use + Clinical trial	strategy assembled from individually approved agents. Amsacrine is not FDA-approved in the United States, which is why US centres substitute other sequential regimens.	Raise sequential conditioning with the transplant programme as a strategy question and ask what their institutional sequential regimen is, given amsacrine is not available in the US

8	Pivekimab sunirine (IMGN632, CD123-directed antibody-drug conjugate)	ADC	Off-label use + Clinical trial	FDA-approved 27 May 2026 as DECNUPAZ (pivekimab sunirine-pvzy) for adults with blastic plasmacytoid dendritic cell neoplasm; boxed warning for hepatotoxicity including hepatic veno-occlusive disease. No AML indication.	Call AbbVie medical information at 844-663-3742 to confirm current off-label supply logistics and to ask whether any AML cohort is reopening
9	Revumenib (SNDX-5613, menin inhibitor)	small_molecule	Off-label use	FDA-approved as REVUFORJ for relapsed or refractory acute leukaemia with a KMT2A translocation (November 2024) and for relapsed or refractory NPM1-mutated AML in adult and paediatric patients 1 year and older (October 2025).	Do not pursue revumenib on the basis of KMT2A amplification; confirm with the treating haematologist that no KMT2A rearrangement was seen on the relapse karyotype
10	Tagraxofusp (SL-401, CD123-directed diphtheria-toxin fusion protein)	other	Off-label use + Clinical trial	FDA-approved (ELZONRIS, blastic plasmacytoid dendritic cell neoplasm, adults and paediatric patients 2 years and older, 2018); EMA-approved for BPDCN. No AML indication.	Email Andrew Lane's team at Dana-Farber NCT03113643) with the CD123 flow result and ask which arm is open, given documented venetoclax refractoriness

11	Venetoclax (BCL-2 inhibition) — prior exposure and TP53-dependent resistance	small_molecule	Off-label use	FDA-approved (VENCLEXTA) including for newly-diagnosed AML in adults 75 or older or unfit for intensive induction, in combination with azacitidine, decitabine or low-dose cytarabine. No approved indication in relapsed or refractory AML.	When screening trials, check whether the assigned arm contains venetoclax and put the documented refractoriness to the investigator before consenting
12	211At-BC8-B10 (astatine-211 anti-CD45) conditioning before allogeneic HCT	radioimmunotherapy	Clinical trial	investigational; no filing	Pull the 2020 conditioning regimen from the transplant record and establish whether it was myeloablative, since that single fact opens or closes this option
13	BMS-986497 (ORM-6151, CD33-directed antibody-conjugated GSPT1 degrader)	ADC	Clinical trial	investigational; IND-stage, no filing	Email Clinical.Trials@bms.com with NCT06419634 in the first line, or call 855-907-3286, and ask which sites have open slots and whether prior allogeneic HCT is permitted
14	CBX-250 (CG1/HLA-A*02:01 x CD3 TCR-mimetic bispecific T-cell engager)	bispecific_other	Clinical trial + Compassionate use	investigational; phase 1, no filing	Retrieve HLA-A*02:01 status first; without it neither this nor the HA-1 route can be screened
15	CD123-directed CAR T cells and switchable CD123 CAR platforms	CAR-T	Clinical trial + Not yet accessible	investigational; no approved CD123 CAR product	Confirm a reachable site before planning anything: at this check the CD123 CAR options are Germany and mainland China only, with no US site
16	CD33-directed CAR T cells	CAR-T	Clinical trial	investigational; no approved CD33 CAR-T product	Confirm a US site is reachable before pursuing the Chinese study: contact City of Hope's clinical trials office about NCT05672147 and ask them to confirm the current screening contact, since the registry record lists no central contact
17	Donor-derived T cells (mLSTs targeting PRAME, WT1, survivin, NY-ESO-1)	other	Clinical trial	investigational; no FDA or EMA filing	Email Marker Therapeutics NCT06552416) now to ask how quickly they can screen a patient who reaches <=10% marrow blasts on salvage, so the window is not missed later

18	Dual CAR T cells (Loop33 x 123 and related constructs)	CAR-T	Clinical trial + Not yet accessible	investigational; no approved dual-targeted CAR product	Confirm whether any US, EU or AU site is accepting patients for a dual CD33/CD123 construct before considering travel to Shenzhen; at this check none was found
19	GTB-3650 (anti-CD16 / IL-15 / anti-CD33 tri-specific killer engager)	other	Clinical trial	investigational; no filing	Call or email Mark Juckett's team (612-624-9452 / juck0001@umn.edu, NCT06594445) and put the post-transplant-therapy question to them directly before anything else
20	HA-1 TCR-T (donor-derived CD8+/CD4+ memory T cells, platform)	other	Clinical trial	investigational; IND-stage, no FDA or EMA filing	Call or email the Fred Hutch immunotherapy intake line (206-606-4668 / immunotherapy@fredhutch.org, NCT03326921) and ask what they need to open a screening file, including whether they will run the HA-1 rs1801284 genotyping in-house for both the patient and the sibling donor
21	Rezatapopt (PC14586, TP53 Y220C-selective p53 reactivator)	small_molecule	Clinical trial + Compassionate use	investigational; no FDA or EMA approval. Sponsor has stated a planned 2027 US submission in TP53 Y220C-mutant ovarian cancer.	Get variant-level TP53 sequencing done on a CLIA assay that reports the specific amino-acid change and the variant allele frequency, and pair it with p53 immunohistochemistry and 17p FISH, since the 2026 panel had a 2% limit of detection and no copy-number calling
22	WT1-siTCR gene-transduced lymphocytes logged upstream as TBI-1301)	other	Not yet accessible	investigational; Japanese academic/industry development, no US or EU filing for the WT1 product	Resolve HLA-A24:02 alongside A02:01 in the same typing pull, so this branch is either opened or closed on data
23	131I-apamistamab (Iomab-B) targeted conditioning before allogeneic HCT	radioimmunotherapy	Unavailable	not approved. The FDA determined that the SIERRA analyses did not adequately support a BLA filing and required an additional randomised study demonstrating an overall-survival benefit (announced August 2024).	Treat Iomab-B as unavailable for this case and put the question to NCT03670966 (211At-BC8-B10) instead

24	Eprenetapopt (APR-246) plus venetoclax and azacitidine	small_molecule	Unavailable	investigational; development discontinued after the phase 3 in TP53-mutant MDS missed its primary endpoint	Record eprenetapopt as closed and do not hold it as a TP53 fallback
25	Flotetuzumab (MGD006, CD123 x CD3 DART)	bispecific_other	Unavailable	investigational; development discontinued, no filing	Record flotetuzumab as closed so it is not re-raised at board
26	GLIDE (guided lymphocyte expansion; donor anti-leukaemia T cells)	other	Unavailable	investigational; academic programme with no filing	If anyone wants this closed definitively, call (514) 252-3400 extension 6247 or email jgnemorin@centrec3i.com and ask whether the protocol is still open
27	HA-1 / HA-2 TCR-T (class review, incl. TSC-100/TSC-101 and BSB-1001)	other	Unavailable	investigational; no FDA or EMA filing for any agent in the class	Treat the TScan and BlueSphere programs as closed for this patient and put the effort into NCT03326921 instead
28	Lintuzumab-Ac225 (225Ac anti-CD33, Actimab-A) after CLAG-M salvage	radioimmunotherapy	Unavailable	investigational; no FDA or EMA filing	If the team wants this pursued, ask Actinium's registry medical contact directly whether isotope supply has a restart timeline
29	MDG1011 (PRAME-directed, autologous TCR-T)	other	Unavailable	investigational; development stopped, no filing	Record PRAME as unavailable rather than pending, so the board does not carry it as a live option
30	VIP943 (CD123-directed antibody-drug conjugate)	ADC	Unavailable	investigational; phase 1, no filing. Sponsor initiated wind-down activities in 2025.	Call one of the named US sites rather than the sponsor first: MD Anderson or Fred Hutch can say in one call whether the VIP943 slot is physically open
31	Vibecotamab (XmAb14045, CD123 x CD3 bispecific)	bispecific_other	Unavailable	investigational; active development ended, no filing	Record vibecotamab as closed
32	WT1-TCRC4 CD8+ T cells (TTCR-C4, prophylactic post-HCT	other	Unavailable	investigational; no FDA or EMA filing	Skip WT1-directed TCR-T for now; the same Fred Hutch intake line (206-606-4668) already being used for NCT03326921 will flag any new WT1 cohort

Standard of care (5)

1. Donor lymphocyte infusion (DLI) from the HLA-identical sibling donor

Aliases: DLI, donor leukocyte infusion, prophylactic DLI

Modality: other · **Regulatory:** Established post-transplant practice; cells are collected and infused under the transplant programme's own authorisations rather than as a licensed product. · **Geographic scope:** United States; DLI itself is available at any transplant centre, the combination study at three sites (St Louis, Pittsburgh, Seattle). · **Verified:** 2026-08-07

DLI is one of the few options in this dossier the patient's own team can arrange without a sponsor, a trial slot or an off-label appeal, since the sibling donor is identified and the relapse pattern points to graft-versus-leukaemia escape rather than graft failure. Its weakness is disease burden, because DLI works poorly against bulky disease and is usually given after cytoreduction. The interferon-gamma combination study would add to it but caps blasts at 30%.

Next steps

1. Ask the original transplant centre what donor cells are still available and whether the sibling would consent to a fresh collection, since that answer takes time to obtain
2. Sequence DLI after cytoreduction rather than against 85% marrow blasts, which is both the clinical and the trial-eligibility position
3. If salvage brings blasts under 30%, contact the UPMC study team (eliaslj@upmc.edu, 412-623-6037, NCT06529731) about the interferon gamma-1b plus DLI protocol
4. Discuss GVHD risk explicitly with the patient, since a six-year disease-free interval suggests a tolerated graft that DLI would deliberately provoke
5. Ask the transplant team about class II HLA downregulation on the relapsed blasts, which is the mechanism most likely to blunt a DLI response after an HLA-identical sibling graft

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06529731	2	recruiting	no	Linda Elias, RN; eliaslj@upmc.edu; 412-623-6037

Manufacturer / sponsor contact

- **Company:** Not applicable; cells come from the original sibling donor via the transplant centre's collection facility
- **Notes:** The only access dependency is the donor's willingness and fitness to donate again, plus the original transplant centre's ability to release or re-collect donor cells.

Payer / coverage: DLI after allogeneic transplant is routinely covered as established care for post-transplant relapse. Costs are the collection, the infusion and the management of any resulting GVHD, all billed through the transplant programme.

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2. Extramedullary myeloid sarcoma (right thigh) — assessment and local control

Aliases: myeloid sarcoma, chloroma, extramedullary AML

Modality: other · **Regulatory:** Not a drug. Biopsy, cross-sectional imaging and local radiotherapy are established clinical services rather than regulated products. · **Geographic scope:** Available at the treating centre; no geographic constraint. · **Verified:** 2026-08-07

This row is biology rather than a drug, and it earns a place because the thigh lesion changes what the other options can achieve. A sanctuary site can carry a different clone from the marrow, so a biopsy of the lesion is the only way to know whether CD33 and CD123 are expressed there at the density the marrow shows. Every trial screening in this dossier will also ask for current extramedullary disease measurements.

Next steps

1. Ask whether the thigh lesion can be biopsied for site-specific immunophenotype and genotype rather than assuming the marrow result applies to it
2. Include the lesion in baseline imaging so response can be assessed separately from the marrow, since trials will ask for it
3. Raise local radiotherapy with radiation oncology for symptom control or consolidation, particularly if a systemic response leaves the thigh lesion behind
4. Flag the extramedullary site to any cell-therapy or engager investigator, since trafficking to soft-tissue disease is a known weak point of those modalities

Manufacturer / sponsor contact

- **Company:** Not applicable
- **Notes:** No manufacturer. Access runs through the treating centre's interventional radiology, pathology and radiation oncology services.

Payer / coverage: Biopsy, MRI and palliative or consolidative radiotherapy to a symptomatic myeloid sarcoma are routinely covered. The item most likely to need justification is repeat cross-sectional imaging for response assessment at a site outside the marrow.

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3. Fractionated gemtuzumab ozogamicin (CD33-directed antibody-drug conjugate)

Aliases: MYLOTARG, CMA-676, gemtuzumab ozogamicin, GO

Modality: ADC · **Regulatory:** FDA-approved (MYLOTARG). The label carries two indications: newly-diagnosed CD33-positive AML in adults and paediatric patients 1 month and older, and relapsed or refractory CD33-positive AML in adults and paediatric patients 2 years and older. EMA-approved for newly-diagnosed CD33-positive AML. · **Geographic scope:** United States and EU for the marketed product; the CLAG-GO study is single-site in Baltimore, Maryland. · **Verified:** 2026-08-07

Gemtuzumab ozogamicin is on-label for exactly this patient's situation, relapsed or refractory CD33-positive AML in an adult, so her own team can prescribe it without a trial slot or an off-label appeal. The open question is antigen density rather than access, since qualitative CD33 positivity does not predict benefit the way antibodies-bound-per-cell does. The CLAG-GO study at Maryland offers the same drug inside a salvage backbone and is the one strong-fit trial in the dossier.

Next steps

1. Order quantitative CD33 antigen-density flow before committing, since it is the variable that separates a reasonable ADC bet from a futile one
2. Contact the University of Maryland study team (veronica.kflu@umm.edu / 410-328-9416, NCT04050280) about a screening slot, and ask how they handle a rapidly rising peripheral blast count given the G-CSF priming
3. If the trial route is slow, have the treating haematologist write fractionated single-agent gemtuzumab on-label as a bridge and confirm the Part B billing pathway locally
4. Document hepatic function and any veno-occlusive-disease risk factors ahead of a possible second transplant, because sinusoidal obstruction syndrome is the sequencing risk
5. Ask Pfizer medical information (1-800-438-1985) for the fractionated MyloFrance-style dosing dossier if the local team wants it

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04050280	2	recruiting	likely	Veronica Kflu; veronica.kflu@umm.edu; 410-328-9416

Manufacturer / sponsor contact

- **Company:** Pfizer Inc. (Wyeth Pharmaceuticals)
- **Med info phone:**1-800-438-1985
- **Product info:**https://www.pfizermedical.com/mylotarg
- **Notes:** Pfizer medical information is staffed 9 am to 5 pm ET Monday to Friday, with online chat and a written-enquiry form on the same site. Pfizer's historic gemtuzumab expanded-access protocol (NCT02312037) is listed as no longer available, which no longer matters now that the product is marketed.

Payer / coverage: This is the unusual case in the dossier where the label already covers the patient's situation: relapsed or refractory CD33-positive AML in an adult. That makes it an on-label Part B buy-and-bill prescription rather than an off-label appeal. Pfizer Oncology Together patient support is reachable at 1-877-744-5675, Monday to Friday 8 am to 8 pm ET.

4. Pending diagnostics that gate the ranked options (HLA class I typing, HA-1/HA-2 genotyping, quantitative CD33 and CD123, TP53 variant-level NGS with CNV, HLA-LOH on blasts)

Aliases: high-resolution HLA typing, HA-1 rs1801284 genotyping, HA-2 genotyping, quantitative antigen density flow, QuantiBRITE ABC, TP53 NGS with copy-number calling, p53 IHC, 17p FISH, HLA loss of heterozygosity

Modality: other · **Regulatory:** Laboratory testing rather than a regulated therapeutic. High-resolution HLA typing runs in ASHI-accredited histocompatibility laboratories; TP53 sequencing, p53 immunohistochemistry, 17p FISH and quantitative flow run in CLIA-certified clinical laboratories. · **Geographic scope:** Available at the treating centre and its reference laboratories, with HA-1/HA-2 genotyping most reliably obtained through the Fred Hutch protocol in Seattle. · **Verified:** 2026-08-07

This is the cheapest and highest-yield item in the whole dossier, and most of it can be ordered today. The 2020 high-resolution HLA typing for both the patient and the sibling almost certainly exists in the first-transplant or donor-registry chart and needs retrieving rather than repeating, and A*02:01 status alone decides whether the HA-1 and CBX-250 routes are open at all. The exceptions are HA-1/HA-2 genotyping, which is not a routine US clinical send-out and is best obtained through the Fred Hutch screening pathway, and quantitative antigen density, which the flow laboratory has to be asked for by name.

Next steps

1. Request the 2020 high-resolution HLA typing reports for the patient and the sibling donor from the transplant centre or donor registry before ordering any new test; retrieval is faster than repeat typing and A*02:01 gates two of the best options
2. Ask the Fred Hutch intake line (206-606-4668 / immunotherapy@fredhutch.org) whether HA-1 rs1801284 and HA-2 genotyping for both the patient and the sibling can be run through NCT03326921 screening, since routine clinical laboratories generally do not offer it
3. Order quantitative CD33 and CD123 antigen density by name (antibodies bound per cell against calibrated beads, not mean fluorescence intensity), because qualitative positivity does not support ADC or engager selection
4. Order TP53 sequencing that reports the specific amino-acid change with variant allele frequency and includes copy-number calling, and add p53 immunohistochemistry and 17p FISH; retrieve the 2019 variant identity and allele frequency at the same time so the 2019-versus-2026 discrepancy is resolved on data
5. Discuss HLA-LOH testing with the transplant team before ordering it: genomic HLA haplotype loss requires a mismatched haplotype, so after an HLA-identical sibling graft the more informative question is HLA class I and class II downregulation on the blasts, which a flow or expression assay answers

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03326921	1	recruiting	unconfirmed	FHCC Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668
NCT06610636	1b	recruiting	unconfirmed	Courtney DiNardo, MD; cdinardo@mdanderson.org; (713) 794-1141

Manufacturer / sponsor contact

- **Company:** Not applicable; hospital and reference laboratories
- **Notes:** No manufacturer contact. The working contacts are the transplant centre's histocompatibility laboratory, the treating centre's flow cytometry laboratory, and the molecular pathology service that ran the 2019 and 2026 panels.

Payer / coverage: High-resolution HLA typing, TP53 sequencing, p53 immunohistochemistry, 17p FISH and diagnostic flow cytometry are routinely covered in a relapsed-leukaemia work-up. Quantitative antigen density and HA-1/HA-2 genotyping are the two that may not have a billing code and may need to run as research or trial-screening tests, which is an argument for routing them through a study team rather than the hospital order set.

5. Second allogeneic haematopoietic cell transplant (HCT2)

Aliases: HCT2, second allograft, salvage allo-HCT

Modality: other · **Regulatory:** Established procedure rather than a regulated product; conditioning agents are individually approved, and treosulfan was FDA-approved in the US in 2025 for use in allogeneic transplant conditioning. · **Geographic scope:** United States; the transplant itself is available at any allogeneic transplant centre, while the two conditioning protocols are Seattle-only. · **Verified:** 2026-08-07

A second allograft is the platform the rest of the curative-intent plan sits on, and unlike most rows here it needs no trial and no manufacturer. What gates it is disease control and a donor decision, not availability. Two Fred Hutch protocols wrap the transplant in a specific conditioning strategy, and both hinge on what the 2020 conditioning regimen was.

Next steps

1. Get a transplant-programme consultation booked now rather than after salvage, since donor work-up and authorisation run on a longer clock than chemotherapy
2. Retrieve the 2020 conditioning regimen and the high-resolution HLA typing for the patient and the sibling, which every transplant protocol here asks for
3. Ask the transplant team to decide between the original HLA-identical sibling and a haploidentical family donor, since some protocols require the latter and graft-versus-leukaemia escape argues for a different donor
4. Contact the Fred Hutch DEC-FLAG-Ida study (nali2@fredhutch.org, 206-667-5854, NCT06928662) if a conditioning trial is wanted, and raise the prior FLAG-Ida failure with them directly
5. Begin payer pre-authorisation early and document the cytoreduction plan, because disease burden is what payers will question

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06928662	1/2	recruiting	unconfirmed	Naveed Ali, MD; nali2@fredhutch.org; 206-667-5854
NCT03670966	1/2	recruiting	unconfirmed	Phuong Vo; ptvo@fredhutch.org; 206-667-2749

Manufacturer / sponsor contact

- **Company:** Not applicable; delivered by transplant centres. Donor search and unrelated-donor logistics run through NMDP.
- **Notes:** Access runs through a transplant programme's evaluation, not a manufacturer. The relevant contacts are the treating centre's transplant coordinator and the original donor's centre.

Payer / coverage: A second allogeneic transplant is covered by Medicare and most commercial plans as established care for relapsed AML, but it is prior-authorised individually and payers look closely at disease control before conditioning. Refractory disease at roughly 85% marrow blasts is the most likely reason for an authorisation fight, which is an argument for documented cytoreduction first.

Off-label use (6)

6. CLAG-GO (cladribine, high-dose cytarabine, G-CSF plus fractionated gemtuzumab ozogamicin)

Aliases: CLAG-GO, CLAG-M + GO, cladribine/cytarabine/G-CSF plus gemtuzumab ozogamicin

Modality: other · **Regulatory:** Regimen of individually approved agents. Gemtuzumab ozogamicin is FDA-approved for relapsed/refractory CD33-positive AML; cladribine, cytarabine and filgrastim are approved products used off-label in this combination. · **Geographic scope:** United States; single trial site in Baltimore, Maryland, with the regimen reproducible at any centre from marketed drugs. · **Verified:** 2026-08-07

This is the one strong-fit trial in the dossier and it takes post-allograft relapse by name. If the Maryland slot does not materialise, the regimen can be reproduced at the home centre from marketed drugs, which is the practical fallback that most other options in this case do not have. The gating items are the 60-day and GVHD conditions, both comfortably met six years out.

Next steps

1. Call the University of Maryland study contacts (410-328-9416 / veronica.kflu@umm.edu) this week and ask about slot availability and screening turnaround for NCT04050280
2. Send the marrow report, current blast trajectory and CD33 flow with the first contact so screening is not delayed
3. Confirm the patient is off all immunosuppression and has no more than grade 1 GVHD, and get that documented
4. Ask the home centre in parallel whether it will deliver CLAG plus fractionated gemtuzumab off-trial if the Baltimore slot slips
5. Plan the cellular or transplant consolidation before starting, since CLAG-GO is a cytoreduction step in a curative-intent sequence rather than an endpoint

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04050280		recruiting	likely	Veronica Kflu; veronica.kflu@umm.edu; 410-328-9416

Manufacturer / sponsor contact

- **Company:** University of Maryland, Baltimore (sponsor); Pfizer supplies gemtuzumab ozogamicin commercially
- **Med info phone:** 1-800-438-1985
- **Product info:** https://clinicaltrials.gov/study/NCT04050280
- **Notes:** Investigator-initiated regimen, so there is no single company to call. The only branded component with a medical-information desk is gemtuzumab ozogamicin (Pfizer).

Payer / coverage: Inside the trial, protocol-specified drug and monitoring are covered by the study. Off-trial, every component is a marketed product and gemtuzumab is on-label here, so the regimen can usually be assembled without a novel coverage fight, though the cladribine-based backbone itself is off-label combination use.

7. FLAMSA-RIC (sequential fludarabine/cytarabine/amsacrine with reduced-intensity conditioning and prophylactic DLI)

Aliases: FLAMSA, FLAMSA-RIC, sequential conditioning

Modality: other · **Regulatory:** Sequential-conditioning strategy assembled from individually approved agents. Amsacrine is not FDA-approved in the United States, which is why US centres substitute other sequential regimens. · **Geographic scope:** Germany for the FLAMSA study; the sequential-conditioning strategy is deliverable at US transplant centres in modified form. · **Verified:** 2026-08-07

FLAMSA-RIC in its original form is hard to reproduce in the United States because amsacrine is not marketed here, and the one recruiting FLAMSA study is in Germany with a venetoclax-containing arm that sits awkwardly against documented venetoclax refractoriness. The transferable idea, cytoreduction running straight into reduced-intensity conditioning, is available in the US through the Fred Hutch DEC-FLAG-Ida protocol or as an institutional sequential regimen. Discuss it as a conditioning strategy with the transplant team rather than as a drug to obtain.

Next steps

1. Raise sequential conditioning with the transplant programme as a strategy question and ask what their institutional sequential regimen is, given amsacrine is not available in the US
2. If the German study is of interest, email Guido Kobbe (kobbe@med.uni-duesseldorf.de, NCT05807932) and ask about non-resident enrollment and about the venetoclax arm in a venetoclax-refractory patient
3. Consider NCT06928662 at Fred Hutch as the US-reachable equivalent, subject to the prior-TBI and 6-month conditions
4. Fold prophylactic post-transplant DLI planning into the conditioning discussion, since it is part of what makes the sequential approach work

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05807932	1/2	recruiting	unconfirmed	Guido Kobbe, Prof. Dr.; kobbe@med.uni-duesseldorf.de; +492118116
NCT06928662	1/2	recruiting	unconfirmed	Naveed Ali, MD; nali2@fredhutch.org; 206-667-5854

Manufacturer / sponsor contact

- **Company:** Not applicable; investigator-directed conditioning strategy using marketed agents
- **Notes:** No single manufacturer. Amsacrine's absence from the US market is the practical reason FLAMSA proper is rarely delivered in the United States.

Payer / coverage: Delivered as part of a transplant admission and authorised with the transplant rather than separately. Agent-level coverage is not usually the obstacle; the transplant authorisation is.

8. Pivekimab sunirine (IMGN632, CD123-directed antibody-drug conjugate)

Aliases: IMGN632, pivekimab sunirine-pvzy, DECNUPAZ, PVEK

Modality: ADC · **Regulatory:** FDA-approved 27 May 2026 as DECNUPAZ (pivekimab sunirine-pvzy) for adults with blastic plasmacytoid dendritic cell neoplasm; boxed warning for hepatotoxicity including hepatic veno-occlusive disease. No AML indication. · **Geographic scope:** United States (FDA approval, May 2026); no equivalent EU approval identified at this check. · **Verified:** 2026-08-07

The BPDCN approval in May 2026 turned pivekimab from a trial-only agent into a drug a treating team can write off-label for CD123-positive AML. Both adult AML studies that took relapsed/refractory patients are closed to new enrollment, and the one recruiting adult study is front-line, so the trial door is effectively shut. The real constraints are payer authorisation and the hepatic veno-occlusive-disease warning ahead of any second allograft.

Next steps

1. Call AbbVie medical information at 844-663-3742 to confirm current off-label supply logistics and to ask whether any AML cohort is reopening
2. Have the transplant team weigh the boxed hepatic veno-occlusive-disease warning against a planned second allo-HCT before the drug is requested, since sequencing is the safety question here
3. Pair the request with quantitative CD123 antigen density rather than qualitative positivity
4. Open the off-label prior-authorisation file early and expect to appeal

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04086264	1/2	active not recruiting	no	—
NCT03386513	1/2	active not recruiting	no	—
NCT07581002	2/3	recruiting	no	ABBVIE CALL CENTER; abbvieclinicaltrials@abbvie.com; 844-663-3742

Manufacturer / sponsor contact

- **Company:** AbbVie (ImmunoGen)
- **Med info phone:**844-663-3742
- **Product info:**https://www.abbviemedinfo.com/
- **Compassionate / expanded access:** https://www.abbvie.com/who-we-are/access-to-investigational-drugs-policy.html
- **Notes:** 844-663-3742 reaches AbbVie medical information (Monday to Friday, 8:00 am to 5:30 pm CT) and is also the number the AbbVie call centre uses on its trial listings. Because the drug is now approved, an AML request is an off-label prescription question rather than a pre-approval-access one; AbbVie's pre-approval policy explicitly does not apply to patients eligible for an ongoing trial.

Payer / coverage: Newly approved for BPDCN, so an AML use is off-label against a 2026 label with a boxed hepatotoxicity warning. Prior authorisation should be expected to be contested, and the veno-occlusive-disease signal matters more than usual in a patient heading toward a second transplant.

9. Revumenib (SNDX-5613, menin inhibitor)

Aliases: SNDX-5613, revumenib, REVUFORJ

Modality: small_molecule · **Regulatory:** FDA-approved as REVUFORJ for relapsed or refractory acute leukaemia with a KMT2A translocation (November 2024) and for relapsed or refractory NPM1-mutated AML in adult and paediatric patients 1 year and older (October 2025). · **Guidelines:** NCCN guidelines list revumenib for relapsed/refractory NPM1-mutated AML. No listing for KMT2A amplification without a translocation. · **Geographic scope:** United States; approved and commercially available. · **Verified:** 2026-08-07

Revumenib is approved and easy to obtain, which makes it tempting, but the label and the biology both turn on a KMT2A translocation or an NPM1 mutation and this patient has neither. KMT2A amplification is a different lesion and does not create menin dependence. The clinician logged it as out of scope for the same reason, and the access answer does not change that.

Next steps

1. Do not pursue revumenib on the basis of KMT2A amplification; confirm with the treating haematologist that no KMT2A rearrangement was seen on the relapse karyotype
2. If a full metaphase karyotype at relapse does turn up a KMT2A translocation, the drug becomes on-label and Syndax medical information (medinfo@syndax.com, 1-888-539-3REV) is the contact
3. Re-test NPM1 at relapse if it has not been done, since clonal evolution could open the second label

Manufacturer / sponsor contact

- **Company:** Syndax Pharmaceuticals, Inc.
- **Med info phone:**1-888-539-3REV (3738)
- **Med info email:**medinfo@syndax.com
- **Product info:**https://syndaxmedical.com/
- **Notes:** Syndax publishes medinfo@syndax.com and 1-888-539-3REV (3738) for medical information and adverse-event reporting. SyndAccess is the company's patient-support and reimbursement programme.

Payer / coverage: On-label use is reimbursed against a KMT2A translocation or an NPM1 mutation. Neither applies here: the 2026 cytogenetics show KMT2A amplification with four copies, which is not a rearrangement, and no NPM1 mutation has been reported. An off-label request without either lesion has no compendia support and should be expected to fail.

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10. Tagraxofusp (SL-401, CD123-directed diphtheria-toxin fusion protein)

Aliases: SL-401, tagraxofusp-erzs, ELZONRIS, DT388IL3

Modality: other · **Regulatory:** FDA-approved (ELZONRIS, blastic plasmacytoid dendritic cell neoplasm, adults and paediatric patients 2 years and older, 2018); EMA-approved for BPDCN. No AML indication. · **Geographic scope:** United States for both the approved product and the open studies; approved in the EU for BPDCN. · **Verified:** 2026-08-07

Tagraxofusp is an approved drug, so a treating team can prescribe it off-label for CD123-positive AML without waiting for a trial slot, but reimbursement is the obstacle rather than supply. The cleaner route is the

Dana-Farber/City of Hope/MD Anderson study, which takes CD123-positive R/R AML and only needs a local CD123 result. Quantitative CD123 density strengthens either route.

Next steps

1. Email Andrew Lane's team at Dana-Farber (andrew_lane@dfci.harvard.edu, NCT03113643) with the CD123 flow result and ask which arm is open, given documented venetoclax refractoriness
2. Order the quantitative CD123 antigen-density flow now; the trial needs a CD123 result within 3 months of first dose and an off-label appeal reads better with density than with a bare 'positive'
3. If the trial route stalls, call Stemline medical information at 1-877-332-7961 for the off-label dossier and have the payer team open a prior-authorisation file early
4. Keep NCT06498973 in reserve as post-HCT2 maintenance, and note its exclusion of post-transplant anti-CD123 therapy when sequencing CD123 agents
5. Flag capillary leak syndrome monitoring requirements to the treating centre before committing, since they shape which site can deliver it

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03113643	1	recruiting	likely	Andrew Lane, MD, PhD; andrew_lane@dfci.harvard.edu; 857-215-1405
NCT06498973	1	recruiting	unconfirmed	Monzr M. Al Malki, MD; malmalki@coh.org; 626-407-1957
NCT06561152	1/2	recruiting	no	Woo In (Yustina) Cho; wooin@stanford.edu; 650-721-2443

Manufacturer / sponsor contact

- **Company:** Stemline Therapeutics, a Menarini Group company
- **Med info phone:** 1-877-332-7961
- **Product info:** https://medical.menarinistemline.com/
- **Notes:** 1-877-332-7961 is the number published for medical information and for reporting adverse events, staffed Monday to Friday 8:30 am to 5:30 pm ET. The grants@menarinistemline.com address on the same site is for grants and sponsorships, not medical enquiries.

Payer / coverage: Off-label in AML. US payers generally hold tagraxofusp to its BPDCN label, so an AML request goes through off-label prior authorisation without an NCCN AML listing behind it; expect to argue it. Enrolling on NCT03113643 sidesteps the coverage question entirely because the drug is supplied.

11. Venetoclax (BCL-2 inhibition) — prior exposure and TP53-dependent resistance

Aliases: ABT-199, GDC-0199, VENCLEXTA, venetoclax

Modality: small_molecule · **Regulatory:** FDA-approved (VENCLEXTA) including for newly-diagnosed AML in adults 75 or older or unfit for intensive induction, in combination with azacitidine, decitabine or low-dose cytarabine. No approved indication in relapsed or refractory AML. · **Geographic scope:** United States and EU; widely available. · **Verified:** 2026-08-07

Venetoclax is easy to get and that is not the issue: this patient progressed through FLAG-IDA plus venetoclax, and TP53 aberration is the canonical driver of BCL-2-inhibitor resistance. The reason this row exists is sequencing rather than acquisition, because several trials on the list carry venetoclax-containing arms that would repeat an exposure she has already failed. Raise that with any investigator before consenting to such an arm.

Next steps

1. When screening trials, check whether the assigned arm contains venetoclax and put the documented refractoriness to the investigator before consenting
2. Do not plan venetoclax re-challenge as monotherapy or with a hypomethylating agent alone on the strength of availability
3. If a mechanistically different combination is proposed, ask what evidence supports re-sensitisation in a TP53-aberrant, previously exposed patient

Manufacturer / sponsor contact

- **Company:** AbbVie and Genentech (co-marketed in the United States)
- **Med info phone:**(800) 821-8590
- **Product info:**https://www.gene.com/medical-professionals/medicines/venclexta
- **Notes:** (800) 821-8590 is the Genentech medical information line, Monday to Friday 5 am to 5 pm PT. AbbVie medical information is 844-663-3742. Genentech's adverse-event line is (888) 835-2555.

Payer / coverage: On-label use is in newly-diagnosed AML in unfit adults; relapsed/refractory use is off-label and, in a patient already documented as refractory to a venetoclax-containing regimen, hard to justify to a payer or to a treating team.

Clinical trial (10)

12. 211At-BC8-B10 (astatine-211 anti-CD45) conditioning before allogeneic HCT

Aliases: 211At-BC8-B10, astatine-211 anti-CD45, At-211 BC8

Modality: radioimmunotherapy · **Regulatory:** investigational; no filing · **Geographic scope:** United States; single site in Seattle, Washington. · **Verified:** 2026-08-07

This is a live, recruiting radioimmunotherapy-conditioning route at Fred Hutch, and it is the practical replacement for Iomab-B. Two facts about this patient decide it: whether the 2020 transplant used myeloablative conditioning, which would exclude her, and whether a haploidentical family donor exists, since the protocol wants a one-haplotype-mismatched related donor rather than the HLA-identical sibling. Both are answerable from the chart and a family history.

Next steps

1. Pull the 2020 conditioning regimen from the transplant record and establish whether it was myeloablative,

since that single fact opens or closes this option

2. Map the family for a haploidentical donor (children, parents, other siblings), because the HLA-identical sibling does not meet this protocol's donor requirement
3. Email Phuong Vo's team (ptvo@fredhutch.org, 206-667-2749, NCT03670966) with the conditioning history and donor picture rather than a general enquiry
4. Confirm CD45 expression on blasts is documented, and get circulating blasts under 10,000/mm3 with hydroxyurea before screening
5. Coordinate with the NCT03326921 intake at the same institution, since both routes run through Fred Hutch and could be sequenced

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03670966	1/2	recruiting	unconfirmed	Phuong Vo; ptvo@fredhutch.org; 206-667-2749

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (sponsor-investigator)
- **Product info:**https://clinicaltrials.gov/study/NCT03670966
- **Notes:** Academic radiochemistry programme with no commercial sponsor or medical-information line; astatine-211 is produced locally, which is why the study is single-site.

Payer / coverage: The conditioning agent is supplied by the study; the transplant itself is billed as clinical care. A haploidentical second transplant needs prior authorisation in its own right, separate from the trial.

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13. BMS-986497 (ORM-6151, CD33-directed antibody-conjugated GSPT1 degrader)

Aliases: ORM-6151, BMS-986497

Modality: ADC · **Regulatory:** investigational; IND-stage, no filing · **Geographic scope:** United States; 16 listed sites, two withdrawn. · **Verified:** 2026-08-07

This is the broadest CD33-directed study open to her on paper: relapsed or refractory AML, any detectable CD33, ECOG up to 2, with prior allograft not named as an exclusion. Sixteen sites makes it geographically the easiest to reach of the CD33 options. The two things to confirm with a site are the post-transplant question and whether the current dose-escalation cohort is open.

Next steps

1. Email Clinical.Trials@bms.com with NCT06419634 in the first line, or call 855-907-3286, and ask which sites have open slots and whether prior allogeneic HCT is permitted
2. Ask for the full protocol exclusion list, since the public registry summary ends with 'other protocol-defined inclusion/exclusion criteria apply'
3. Send documented CD33 expression with the enquiry and add quantitative density when it returns
4. Check whether the marrow blast burden and any leukocytosis limits apply, as they are not in the public record

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06419634	1	recruiting	likely	BMS Clinical Trials Contact Center www.BMSClinicalTrials.com; Clinical.Trials@bms.com; 855-907-3286

Manufacturer / sponsor contact

- **Company:** Bristol-Myers Squibb (Orum Therapeutics originated ORM-6151)
- **Med info phone:**1-800-321-1335
- **Med info email:**medical.communications@bms.com
- **Product info:**https://www.bmsmedinfo.com/
- **Notes:** 1-800-321-1335 is the US medical information contact centre, Monday to Friday 8 am to 5 pm ET. BMS publishes pre-approval access and post-study access information through its healthcare-provider medical resources pages.

Payer / coverage: Investigational; drug supplied by the sponsor. Standard trial-participation cost rules apply.

14. CBX-250 (CG1/HLA-A*02:01 x CD3 TCR-mimetic bispecific T-cell engager)

Aliases: CBX-250, CG1/HLA-A2 TCR-mimetic, CROSSCHECK-001

Modality: bispecific_other · **Regulatory:** investigational; phase 1, no filing · **Geographic scope:** United States; eleven listed sites across CA, FL, IL, MA, MO, NY and PA. · **Verified:** 2026-08-07

Of the HLA-A*02:01-restricted options, this is the one with the widest geographic footprint and the most permissive post-transplant language, and it also has a published compassionate-use address if a slot is not available. The single decisive gate is HLA-A*02:01 status, the same value that governs the HA-1 pathway. White cells need to be under 25,000 at enrollment, which cytoreduction can address.

Next steps

1. Retrieve HLA-A*02:01 status first; without it neither this nor the HA-1 route can be screened
2. Email Rachel Ghiraldi (rachel.ghiraldi@crossbowtx.com, 857-301-6432, NCT06994676) to ask which of the eleven sites has open slots and what cytoreduction they accept to reach a white count under 25,000
3. Ask whether CG1 (cathepsin G) expression is measured on study or whether HLA-A*02:01 positivity alone qualifies, so the workup list is complete
4. If no slot is available, have the treating physician request the compassionate-use form at clinical@crossbowtx.com and submit it alongside the trial enquiry rather than after it
5. Note the ECOG 0-1 requirement and keep performance status documented, since it can drift quickly at this disease burden

Trial pathways

NCT	Phase	Status	Eligible	Central contact

NCT06994676	1	recruiting	likely	Rachel Ghiraldi; rachel.ghiraldi@crossbowtx.com; 857-301-6432

Manufacturer / sponsor contact

- **Company:** Crossbow Therapeutics, Inc.
- **Med info phone:**857-301-6432
- **Med info email:**rachel.ghiraldi@crossbowtx.com
- **Product info:**https://www.crossbowtx.com/
- **Compassionate use email:**clinical@crossbowtx.com
- **Notes:** Crossbow publishes a compassionate-use policy stating that physicians interested in treating a patient with CBX-250 should complete the request form and send it to clinical@crossbowtx.com. That gives this agent something most phase 1 programmes in this dossier lack: a named route for a patient who cannot enrol.

Payer / coverage: Sponsor-supplied agent inside the trial. A compassionate-use supply would still leave the patient's insurer covering administration, monitoring and cytokine-release management.

15. CD123-directed CAR T cells and switchable CD123 CAR platforms

Aliases: CD123 CAR-T, Allo-RevCAR01-T, R-TM123, UniCAR

Modality: CAR-T · **Regulatory:** investigational; no approved CD123 CAR product · **Geographic scope:** Germany (11 sites) and mainland China (1 site). No US, UK or AU site identified. · **Verified:** 2026-08-07

The switchable CD123 CAR platform is the most interesting of the CD123 cell therapies because the target module can be switched off if toxicity develops, but its blast ceiling of about 30% puts it out of reach at 85% marrow blasts. It is also Germany-only. If a cytoreductive regimen produces a real response, this becomes worth a second look; until then it is closed on disease burden as much as on geography.

Next steps

1. Confirm a reachable site before planning anything: at this check the CD123 CAR options are Germany and mainland China only, with no US site
2. If salvage brings marrow blasts under 30%, email AVC-201-01@avencell.com to ask whether a US-resident patient can be screened at a German site and what the residency and follow-up requirements are
3. Order quantitative CD123 antigen density, since the AvenCell protocol states that exceptions to minimum CD123 expression are not permitted
4. Prefer reachable CD123 agents (tagraxofusp, pivekimab) for cytoreduction rather than holding a slot open for a CAR that cannot enroll her today

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05949125	1	recruiting	no	Katja Jersemann, Dr.; AVC-201-01@avencell.com; 0493514466450

NCT04010877	1/2	recruiting	unconfirmed	Lung-Ji Chang, Ph.D; c@szgimi.org; +86 0755-86573763

Manufacturer / sponsor contact

- **Company:** AvenCell Europe GmbH (Allo-RevCAR01-T); Shenzhen Geno-Immune Medical Institute
- **Med info phone:**0493514466450
- **Med info email:**AVC-201-01@avencell.com
- **Notes:** The AvenCell address published on the registry record is study-specific rather than a general medical-information desk. No expanded-access policy identified for either sponsor.

Payer / coverage: Both routes sit outside US coverage. On the German study the agent is sponsor-supplied but everything around it, including travel and any bridging therapy, falls outside a US plan's network.

16. CD33-directed CAR T cells

Aliases: CART-33, anti-CD33 CAR-T, CD33 CAR T

Modality: CAR-T · **Regulatory:** investigational; no approved CD33 CAR-T product · **Geographic scope:** United States (City of Hope, Duarte CA) and mainland China (Shenzhen) only. · **Verified:** 2026-08-07

The City of Hope study is the realistic CD33 CAR-T route and it is written for a post-allograft patient with a donor still identifiable, which describes this case. On-target marrow aplasia means the protocol assumes a subsequent transplant, so it only makes sense as part of a transplant-consolidation plan. The Shenzhen study is open but mainland-China-only.

Next steps

1. Confirm a US site is reachable before pursuing the Chinese study: contact City of Hope's clinical trials office about NCT05672147 and ask them to confirm the current screening contact, since the registry record lists no central contact
2. Confirm the sibling donor's availability and willingness for a further collection, because the protocol requires the original donor as the stem-cell source
3. Order quantitative CD33 antigen density alongside the trial enquiry
4. Ask the study team how they sequence CD33 CAR-T against a planned second allograft, since the aplasia risk drives the timeline
5. Only revisit NCT04010877 if the US route closes, and then ask explicitly about non-resident enrollment and follow-up obligations in Shenzhen

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05672147	1	recruiting	likely	—
NCT04010877	1/2	recruiting	unconfirmed	Lung-Ji Chang, Ph.D; c@szgimi.org; +86 0755-86573763

Manufacturer / sponsor contact

- **Company:** City of Hope Medical Center (sponsor, NCT05672147); Shenzhen Geno-Immune Medical Institute (sponsor, NCT04010877)
- **Notes:** Academic sponsors; no commercial medical-information line and no published expanded-access programme for either construct.

Payer / coverage: Cell product is supplied by the study. Lymphodepletion, apheresis, prolonged cytopenia management and the planned consolidative transplant are billed to the insurer, and CD33 CAR-T carries an expectation of marrow aplasia that makes the transplant backup non-optional.

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17. Donor-derived multi-leukemia-antigen-specific T cells (mLSTs targeting PRAME, WT1, survivin, NY-ESO-1)

Aliases: multiTAA-specific T cells, mLSTs, TAA-T, ADSPAM, MT-401-OTS

Modality: other · **Regulatory:** investigational; no FDA or EMA filing · **Geographic scope:** United States; MT-401-OTS at City of Hope, Moffitt and KU Cancer Center. · **Verified:** 2026-08-07

Multi-antigen T-cell products are all gated on low disease burden, and at 85% marrow blasts none of the three studies will take her now. MT-401-OTS is the one worth tracking, because it becomes reachable if a salvage regimen drops marrow blasts to 10% or less. The other two are either closed to enrollment or written for patients who have not relapsed.

Next steps

1. Email Marker Therapeutics (pallison@markertherapeutics.com, NCT06552416) now to ask how quickly they can screen a patient who reaches <=10% marrow blasts on salvage, so the window is not missed later
2. Sequence this behind whatever cytoreductive option the board picks; it is a consolidation-slot candidate, not an induction one
3. Ask whether the off-the-shelf product requires any HLA restriction the patient must be typed for, and fold that into the same typing request
4. Re-check NCT02494167 status in three months in case Baylor reopens accrual

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06552416	1	recruiting	no	Patricia Allison, BS; pallison@markertherapeutics.com; 713-400-6400
NCT02494167	1	active not recruiting	no	—
NCT02203903	1	recruiting	no	Fahmida Hoq, MBBS, MS; fhoq@childrensnational.org; 202-476-3634

Manufacturer / sponsor contact

- **Company:** Marker Therapeutics (MT-401-OTS); Baylor College of Medicine and Children's National (academic

programs)
- **Med info phone:**713-400-6400
- **Med info email:**pallison@markertherapeutics.com
- **Notes:** Marker publishes no separate expanded-access policy; the registry study contacts are the working route.

Payer / coverage: Investigational cell products are sponsor-supplied. The cost exposure is standard-of-care bridging therapy and travel, not the agent.

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18. Dual CD33/CD123-directed CAR T cells (Loop33 x 123 and related constructs)

Aliases: Loop33x123, CD33/CD123 dual CAR, CLL-1/CD33/CD123 CAR-T

Modality: CAR-T · **Regulatory:** investigational; no approved dual-targeted CAR product · **Geographic scope:** Mainland China only (Shenzhen, Guangdong). No US, EU or AU site identified. · **Verified:** 2026-08-07

Dual CD33/CD123 targeting exists in the clinic only through a single mainland-China study, so this is a geography problem before it is a biology problem. The dual-antigen rationale is strong for a patient whose blasts carry both antigens, but no US or EU site is running it. Both antigens can still be attacked sequentially with agents that are reachable.

Next steps

1. Confirm whether any US, EU or AU site is accepting patients for a dual CD33/CD123 construct before considering travel to Shenzhen; at this check none was found
2. If the family wants the Chinese option explored anyway, have the treating haematologist write to c@szgimi.org about non-resident enrollment, manufacturing timelines and follow-up obligations, and get the cost structure in writing
3. In the meantime pursue CD33 and CD123 sequentially through reachable agents rather than waiting on a dual construct
4. Order quantitative CD33 and CD123 density, which any dual-targeting conversation will require

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04010877	1/2	recruiting	unconfirmed	Lung-Ji Chang, Ph.D; c@szgimi.org; +86 0755-86573763

Manufacturer / sponsor contact

- **Company:** Shenzhen Geno-Immune Medical Institute
- **Notes:** Academic institute; no published expanded-access route and no medical-information line identified. Correspondence contact is the principal investigator listed on the registry record.

Payer / coverage: US insurers do not cover care delivered on a Chinese academic protocol. Cell manufacture, hospitalisation and toxicity management would be self-funded, and the CAR-T toxicity window is not one to manage far from the treating team.

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19. GTB-3650 (anti-CD16 / IL-15 / anti-CD33 tri-specific killer engager)

Aliases: GTB-3650, cam1615CD33, CD33 TriKE

Modality: other · **Regulatory:** investigational; no filing · **Geographic scope:** United States; single site in Minneapolis, Minnesota. · **Verified:** 2026-08-07

A single-site phase 1 in Minneapolis with a live contact, but eligibility hinges on a clause that reads against this patient: transplant candidate, or newly relapsed after transplant with no post-transplant therapy given. Since she has already had FLAG-IDA plus venetoclax since relapsing, the study team has to rule on it rather than the file. Peripheral blasts also need to be under 20,000 at treatment start.

Next steps

1. Call or email Mark Juckett's team (612-624-9452 / juck0001@umn.edu, NCT06594445) and put the post-transplant-therapy question to them directly before anything else
2. Ask whether hydroxyurea-controlled blasts under 20,000 is acceptable given a peripheral blast fraction that reached 82%
3. Confirm the inpatient or infusion-schedule commitment and expected duration, since this is a travel-dependent option
4. Keep this behind CLAG-GO and the CD33 ADC routes unless the eligibility ruling comes back favourable

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06594445		recruiting	unconfirmed	Mark Juckett, MD; juck0001@umn.edu; 612-624-9452

Manufacturer / sponsor contact

- **Company:** GT Biopharma, Inc. (product); Masonic Cancer Center, University of Minnesota (sponsor)
- **Notes:** No manufacturer medical-information line captured. The academic sponsor's study contact is the working route.

Payer / coverage: Sponsor-supplied agent; a continuous-infusion schedule means an inpatient or infusion-centre commitment that should be confirmed with the site before travel is booked.

20. HA-1 TCR-T (donor-derived CD8+/CD4+ memory T cells, Bleakley/HighPassBio platform)

Aliases: HA-1 TCR T, HA-1 T TCR T cells, FH HA-1 TCR-T, Bleakley HA-1 TCR-T, HPB-101

Modality: other · **Regulatory:** investigational; IND-stage, no FDA or EMA filing · **Geographic scope:** United States; single open site (Seattle, WA). Unrelated donors residing outside the US are handled through NMDP-affiliated centres, which does not apply to an HLA-identical sibling. · **Verified:** 2026-08-07

The Fred Hutch phase 1 is the one HA-1-directed program written for a patient like this one: prior allo-HCT is required, not excluded, and 85% marrow blasts does not disqualify. Everything now turns on genotyping, since the recipient must be HLA-A*02:01 positive and HA-1(H) positive while the sibling donor must be HA-1(H)-negative or A*02:01-negative. There is no off-trial route to this product.

Next steps

1. Call or email the Fred Hutch immunotherapy intake line (206-606-4668 / immunotherapy@fredhutch.org, NCT03326921) and ask what they need to open a screening file, including whether they will run the HA-1 rs1801284 genotyping in-house for both the patient and the sibling donor
2. Retrieve the 2020 high-resolution HLA typing for the patient and the sibling from the first-transplant / donor-registry chart before the call, since A*02:01 status decides whether the referral is worth making
3. Confirm with the sibling donor that a repeat apheresis collection is acceptable to her, because the product is donor-derived
4. Ask the study team about current slot availability and expected screening-to-infusion interval, and whether bridging cytoreduction on another protocol is permitted in the interval
5. Have the referring centre start out-of-network / travel authorisation in parallel rather than after eligibility is confirmed

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03326921		recruiting	unconfirmed	FHCC Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (sponsor-investigator); HA-1 TCR originated with Marie Bleakley at Fred Hutch and was licensed to HighPassBio, an ElevateBio company
- **Product info:**https://clinicaltrials.gov/study/NCT03326921
- **Notes:** No commercial medical-information line exists for this product. Cell manufacture and supply sit with the Fred Hutch program, so the trial intake line is the only route in.

Payer / coverage: Trial-supplied cell product; the sponsor covers the investigational agent. Routine care costs, lymphodepletion, donor collection and travel to Seattle are billed to the patient's insurer, and out-of-network authorisation for a Washington-state referral is the usual friction point.

21. Rezatapopt (PC14586, TP53 Y220C-selective p53 reactivator)

Aliases: PC14586, rezatapopt, PYNNACLE

Modality: small_molecule · **Regulatory:** investigational; no FDA or EMA approval. Sponsor has stated a planned 2027 US submission in TP53 Y220C-mutant ovarian cancer. · **Geographic scope:** United States; the haematologic study runs at MD Anderson (Houston, TX) and Moffitt (Tampa, FL). · **Verified:** 2026-08-07

Rezatapopt is only worth pursuing if the TP53 variant turns out to be Y220C, and the trial requires that confirmed on a CLIA assay above 2% variant allele frequency, which this case does not have. The one haematologic study runs at MD Anderson and Moffitt with a named contact. PMV also publishes an expanded-access address, so a Y220C-positive result opens two doors rather than one.

Next steps

1. Get variant-level TP53 sequencing done on a CLIA assay that reports the specific amino-acid change and the variant allele frequency, and pair it with p53 immunohistochemistry and 17p FISH, since the 2026 panel had a 2% limit of detection and no copy-number calling
2. If Y220C is confirmed, email Courtney DiNardo's team at MD Anderson (cdinardo@mdanderson.org, NCT06616636) with the report attached
3. If Y220C is confirmed but no slot exists, have the treating physician submit an expanded-access request to clinicaltrials@pmvpharma.com per PMV's published policy
4. If the variant is a non-Y220C TP53 aberration, close this line rather than substituting eprenetapopt, which has no live programme
5. Retrieve the 2019 report's exact variant identity and allele frequency in parallel; it may answer the question without new sequencing

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06616636	1b	recruiting	unconfirmed	Courtney DiNardo, MD; cdinardo@mdanderson.org; (713) 794-1141
NCT04585750	1/2	recruiting	no	PMV Pharma Clinical Study Information Center; clinicaltrials@pmvpharma.com; (609) 235-4038

Manufacturer / sponsor contact

- **Company:** PMV Pharmaceuticals, Inc.
- **Med info phone:**(609) 235-4038
- **Med info email:**clinicaltrials@pmvpharma.com
- **Compassionate / expanded access:**https://www.pmvpharma.com/pipeline/access/
- **Compassionate use email:**clinicaltrials@pmvpharma.com
- **Notes:** PMV's published access policy states that a treating physician may submit an inquiry or a formal expanded-access request on a patient's behalf by email to clinicaltrials@pmvpharma.com, with each request evaluated case by case.

Payer / coverage: Investigational and sponsor-supplied on study. An expanded-access supply is typically provided at no charge but does not cover the surrounding care.

Not yet accessible (1)

22. WT1-siTCR gene-transduced lymphocytes (HLA-A*24:02-restricted; logged upstream

as TBI-1301)

Aliases: WT1-siTCR, TBI-1301 (as logged upstream), TBI-1201, Mie University WT1 siTCR

Modality: other · **Regulatory:** investigational; Japanese academic/industry development, no US or EU filing for the WT1 product · **Geographic scope:** Japan; no US, EU or AU site for the WT1 product. · **Verified:** 2026-08-07

The WT1-siTCR platform sits with Japanese investigators and has no open study a US patient could enter, so this is a geography-and-sponsor problem rather than a biology one. It only becomes a question at all if the patient turns out to be HLA-A*02:01-negative and A*24:02-positive. Even then the route would be an investigator-initiated Japanese protocol.

Next steps

1. Resolve HLA-A*24:02 alongside A*02:01 in the same typing pull, so this branch is either opened or closed on data
2. If A*02:01-negative and A*24:02-positive, ask the treating haematologist to write to the Mie University group about any enrolling WT1-siTCR study before assuming there is one
3. Confirm the product designation before any outreach: upstream files carry this as TBI-1301, which is Takara's NY-ESO-1 siTCR product, so the WT1 program identifier should be checked against the sponsor's own materials

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02550535	1/2	completed	no	—

Manufacturer / sponsor contact

- **Company:** Takara Bio Inc. / Mie University (Japan)
- **Notes:** No US medical-information line for the WT1 siTCR product. Takara's active siTCR clinical work is the NY-ESO-1 product (mipetresgene autoleucel), for which a new Japanese confirmatory trial notification cleared PMDA review in September 2025; that is a different antigen and a different indication.

Notes: Possible upstream naming drift: TBI-1301 is Takara's NY-ESO-1 siTCR product (mipetresgene autoleucel), while the WT1 siTCR program has been reported under a separate designation. Flagged rather than corrected, since trials.jsonl and the evidence files are not mine to edit.

Unavailable (10)

23. 131I-apamistamab (Iomab-B) targeted conditioning before allogeneic HCT

Aliases: Iomab-B, 131I-apamistamab, BC8, anti-CD45 radioimmunotherapy

Modality: radioimmunotherapy · **Regulatory:** not approved. The FDA determined that the SIERRA analyses did not adequately support a BLA filing and required an additional randomised study demonstrating an overall-survival benefit (announced August 2024). · **Geographic scope:** No accessible path; the sponsor's new study has no listed sites and the European commercialisation agreement is contingent on a future approval. · **Verified:** 2026-08-07

Iomab-B has no approval anywhere and the FDA rejected the SIERRA package, asking for a new randomised trial. That trial is not yet recruiting, has no listed sites, excludes anyone with a prior allograft and caps marrow blasts at 20%, so it fails this patient on two independent criteria. Targeted radioimmunotherapy conditioning stays on the table only through the astatine-211 anti-CD45 protocol at Fred Hutch.

Next steps

1. Treat Iomab-B as unavailable for this case and put the radioimmunotherapy-conditioning question to NCT03670966 (211At-BC8-B10) instead
2. If confirmation is wanted, email mvusirikala@actiniumpharma.com asking whether any prior-transplant cohort is planned for the new study
3. Do not delay a second-transplant decision waiting for this programme, which is years from any approval

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07157514	2/3	not yet recruiting	no	Madhuri Vusirikala, MD; mvusirikala@actiniumpharma.com; 347-814-2268
NCT02665065	3	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Actinium Pharmaceuticals, Inc.
- **Product info:**https://www.actiniumpharma.com/
- **Notes:** No medical-information or expanded-access line is published on Actinium's contact page, which lists investorrelations@actiniumpharma.com and a New York address. The registry medical contact for the new study (mvusirikala@actiniumpharma.com, 347-814-2268) is a trial contact. Actinium and Immedica announced a commercialisation agreement for Iomab-B in Europe, the Middle East and North Africa, which is contingent on an approval that has not happened.

24. Eprenetapopt (APR-246) plus venetoclax and azacitidine

Aliases: APR-246, PRIMA-1Met, eprenetapopt

Modality: small_molecule · **Regulatory:** investigational; development discontinued after the phase 3 in TP53-mutant MDS missed its primary endpoint · **Geographic scope:** No open access path in any geography. · **Verified:** 2026-08-07

The registrational trial in TP53-mutant MDS failed its primary endpoint and Aprea moved on, so there is no sponsor, no open study and no compassionate route. The upstream read that eprenetapopt is low-yield here matches the access picture. If the TP53 question resolves to Y220C, rezatapopt is the live molecule, not this one.

Next steps

1. Record eprenetapopt as closed and do not hold it as a TP53 fallback
2. Direct any confirmed TP53 Y220C result to rezatapopt (NCT06616636) instead

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04214860	1	completed	no	—

Manufacturer / sponsor contact

- **Company:** Aprea Therapeutics, Inc.
- **Notes:** After the phase 3 miss the company acquired Atrin and moved its focus to DNA-damage-response assets. No current eprenetapopt trials and no expanded-access programme identified.

25. Flotetuzumab (MGD006, CD123 x CD3 DART)

Aliases: MGD006, S80880, flotetuzumab

Modality: bispecific_other · **Regulatory:** investigational; development discontinued, no filing · **Geographic scope:** No open access path in any geography. · **Verified:** 2026-08-07

MacroGenics stopped flotetuzumab on a business decision and then closed its expanded-access protocol, so both the trial and the compassionate routes are gone. The CD123 x CD3 engager idea survives in other molecules, not in this one. No action is available here.

Next steps

1. Record flotetuzumab as closed so it is not re-raised at board
2. Carry the CD123 engager rationale into agents that still have a sponsor rather than pursuing this molecule

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02152956	1/2	terminated	no	—
NCT04678466	—	no longer available	no	—

Manufacturer / sponsor contact

- **Company:** MacroGenics, Inc.
- **Notes:** No medical-information line captured for a discontinued programme; the company's registry study director listing is historical.

26. GLIDE (guided lymphocyte immunopeptide-derived expansion; donor anti-leukaemia T cells)

Aliases: GLIDE, guided lymphocyte immunopeptide derived expansion

Modality: other · **Regulatory:** investigational; academic programme with no filing · **Geographic scope:** Canada; single site in Montreal, Quebec. · **Verified:** 2026-08-07

The GLIDE record has sat untouched since 2017 with a completion date in 2019 and a registry status of unknown, which in practice means the study is not enrolling. One phone call would settle it, but nothing here should be planned around it. Note the site is in Montreal, so a Canadian referral would be needed even if it were live.

Next steps

1. If anyone wants this closed definitively, call (514) 252-3400 extension 6247 or email jgnemorin@centrec3i.com and ask whether the protocol is still open
2. Otherwise treat GLIDE as dormant and do not carry it into the ranking as a live option
3. Note the Canadian location, which would require a cross-border referral even in the best case

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03091933	1/2	unknown	unconfirmed	Jean-Guy Némorin, PhD; jgnemorin@centrec3i.com; (514) 252-3400

Manufacturer / sponsor contact

- **Company:** CIUSSS de l'Est-de-l'Ile-de-Montreal (Hopital Maisonneuve-Rosemont)
- **Notes:** Academic sponsor; no commercial contact. Any enquiry goes to the listed study contacts, whose currency after nine years is unverified.

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27. HA-1 / HA-2 minor-histocompatibility-antigen TCR-T (class review, incl. TSC-100/TSC-101 and BSB-1001)

Aliases: TSC-100, TSC-101, BSB-1001, HA-1 TCR-T, HA-2 TCR-T, MDG1021

Modality: other · **Regulatory:** investigational; no FDA or EMA filing for any agent in the class · **Geographic scope:** United States (both programs); no EU sites listed. · **Verified:** 2026-08-07

Every commercial HA-1/HA-2 TCR-T program is built around a first transplant and excludes patients who have already had an allograft, which this patient had in 2020. TScan states it does not supply outside trials, so there is no compassionate-use fallback either. The HA-directed idea stays alive for her only through the Fred Hutch protocol (NCT03326921), which requires prior allo-HCT.

Next steps

1. Treat the TScan and BlueSphere programs as closed for this patient and put the effort into NCT03326921 instead
2. If the treating team wants the exclusion confirmed in writing, email medicalaffairs@tscan.com quoting NCT05473910 and the prior matched-sibling allograft
3. Ask BlueSphere (nkhan@bluespherebio.com) whether any post-transplant-relapse cohort of BSB-1001 is planned, and log the answer for a future re-screen
4. Re-screen this class if the patient proceeds to a second allograft, since a first-transplant-conditioned design could become relevant to a subsequent donor pairing

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05473910	1	recruiting	no	Tim White; medicalaffairs@tscan.com; (857) 399-9500
NCT06704152	1/2	recruiting	no	Medical Director: Nawazish Khan, MD, BlueSphere Bio; nkhan@bluespherebio.com; 252-347-4938
NCT04464889	1	withdrawn	no	—

Manufacturer / sponsor contact

- **Company:** TScan Therapeutics (TSC-100/TSC-101); BlueSphere Bio (BSB-1001)
- **Med info phone:**(857) 399-9500
- **Med info email:**medicalaffairs@tscan.com
- **Compassionate / expanded access:**https://www.tscan.com/for-patients/expanded-access-policy/
- **Notes:** TScan's published expanded-access policy states it does not provide access to investigational products outside clinical trials, and directs provider questions to medicalaffairs@tscan.com. BlueSphere Bio publishes no expanded-access policy; the registry medical-director contact is the only route.

28. Lintuzumab-Ac225 (225Ac anti-CD33, Actimab-A) after CLAG-M salvage

Aliases: Actimab-A, 225Ac-lintuzumab, lintuzumab-Ac225, HuM195-Ac225

Modality: radioimmunotherapy · **Regulatory:** investigational; no FDA or EMA filing · **Geographic scope:** United States; both suspended studies are NCI-network. · **Verified:** 2026-08-07

Both NCI studies of lintuzumab-Ac225 are suspended with 'Drug Supply Issues' on the registry record, and no restart date is posted. The CLAG-M combination that generated the salvage rationale completed years ago. Until actinium-225 supply is resolved there is nothing to enrol into and no compassionate protocol to request.

Next steps

1. If the team wants this pursued, ask Actinium's registry medical contact (mvusirikala@actiniumpharma.com) directly whether isotope supply has a restart timeline
2. Do not build a treatment plan around a suspended isotope programme; keep CLAG-M itself as the

transferable part of the rationale
3. Re-check both NCI records in 8 to 12 weeks, since supply suspensions can lift without an announcement

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06802523	1/2	suspended	no	—
NCT06881823	1/2	suspended	no	—
NCT03441048	1	completed	no	—

Manufacturer / sponsor contact

- **Company:** Actinium Pharmaceuticals, Inc.
- **Product info:**https://www.actiniumpharma.com/
- **Notes:** Actinium's public contact page lists investorrelations@actiniumpharma.com and a New York address but no medical-information or expanded-access line. The nearest verifiable clinical contact is the Actinium medical contact listed on the apamistamab registry record (mvusirikala@actiniumpharma.com, 347-814-2268), which is a trial contact rather than a medical-information desk.

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29. MDG1011 (PRAME-directed, HLA-A*02:01-restricted autologous TCR-T)

Aliases: MDG1011, PRAME TCR-T, BPX-701 (related PRAME TCR-T)

Modality: other · **Regulatory:** investigational; development stopped, no filing · **Geographic scope:** No open access path in any geography. · **Verified:** 2026-08-07

Both PRAME TCR-T programs that reached patients are closed, and Medigene is in insolvency proceedings, so there is no sponsor left to ask. PRAME as a target is not dead, but no PRAME-directed cell product is obtainable for this patient today. Do not spend screening effort here.

Next steps

1. Record PRAME as unavailable rather than pending, so the board does not carry it as a live option
2. If PRAME expression is measured for other reasons, keep the result on file for any future PRAME-directed engager or TCR program
3. Re-screen only if a new sponsor announces a PRAME TCR-T study that accepts post-allo-HCT relapse

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03503968	1/2	terminated	no	—
NCT02743611	1/2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Medigene AG (MDG1011); Bellicum Pharmaceuticals (BPX-701)
- **Notes:** The Munich local court opened insolvency proceedings over the assets of Medigene AG and Medigene Immunotherapies GmbH by order dated 1 August 2025, after the company had already shifted away from haematology and sought a partner for MDG1011. There is no sponsor to receive a compassionate-use request.

30. VIP943 (CD123-directed antibody-drug conjugate)

Aliases: VIP943, VIP-943

Modality: ADC · **Regulatory:** investigational; phase 1, no filing. Sponsor initiated wind-down activities in 2025. · **Geographic scope:** United States; five listed sites, all of unverified current status. · **Verified:** 2026-08-07

The registry entry looks open but the sponsor does not: Vincerx lost two merger attempts in 2025 and its board authorised wind-down while seeking to out-license assets, and the trial record has been untouched since November 2024 with its completion date passed. Reporting this as cleanly recruiting would be wrong. The honest position is that VIP943 is unavailable unless a site says otherwise on the phone.

Next steps

1. Call one of the named US sites rather than the sponsor first: MD Anderson or Fred Hutch can say in one call whether the VIP943 slot is physically open
2. Try clinicaltrials@vincerx.com / 16508006676 as captured from the registry, understanding that the line may not be staffed
3. If a site confirms closure, treat CD123 targeting through tagraxofusp or pivekimab instead
4. Watch for an out-licensing announcement, which would be the only route by which this asset returns

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06034275		recruiting	unconfirmed	Vincerx Clinical Trials Contact; clinicaltrials@vincerx.com; 16508006676

Manufacturer / sponsor contact

- **Company:** Vincerx Pharma, Inc.
- **Med info phone:**16508006676
- **Med info email:**clinicaltrials@vincerx.com
- **Notes:** The email and phone are captured verbatim from the registry record and may no longer be staffed given the wind-down. Treat both as unverified for liveness.

Notes: Flagged upstream and confirmed here: recruitment status on the registry is stale rather than current.

31. Vibecotamab (XmAb14045, CD123 x CD3 bispecific)

Aliases: XmAb14045, vibecotamab

Modality: bispecific_other · **Regulatory:** investigational; active development ended, no filing · **Geographic scope:** No open access path in any geography. · **Verified:** 2026-08-07

Vibecotamab has no sponsor advancing it and the last academic study closed in May 2026. There is no trial, no expanded-access protocol and no marketed product to prescribe. Treat it as a closed line of enquiry.

Next steps

1. Record vibecotamab as closed
2. If a CD123 x CD3 engager is wanted, direct the question to sponsors with live programmes rather than to Xencor

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05285813	2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Xencor, Inc.
- **Notes:** Novartis returned ex-US rights and Xencor did not carry the molecule forward, so there is no sponsor supplying it. No medical-information line captured for a shelved programme.

32. WT1-TCRC4 CD8+ T cells (TTCR-C4, HLA-A*02:01-restricted), prophylactic post-HCT

Aliases: TTCR-C4, WT1-TCRc4, WT1 TCR C4, FH-WT1-E50 (successor)

Modality: other · **Regulatory:** investigational; no FDA or EMA filing · **Geographic scope:** United States; Seattle only. · **Verified:** 2026-08-07

The original WT1-TCRc4 protocol closed for slow accrual and nothing replaced it for post-transplant relapse. The live Fred Hutch WT1 successor treats MRD in first remission and excludes patients with a prior allograft, so neither protocol is open to this patient. WT1 stays a target of interest only if a future study accepts post-HCT relapse.

Next steps

1. Skip WT1-directed TCR-T for now; the same Fred Hutch intake line (206-606-4668) already being used for NCT03326921 will flag any new WT1 cohort
2. Ask the intake team directly whether a post-allo-HCT relapse cohort of FH-WT1-E50 is planned, and record the answer with a date
3. Keep HLA-A*02:01 status on file, since it gates any future WT1 protocol as well as the HA-1 one

Trial pathways

NCT	Phase	Status	Eligible	Central contact

NCT01640301	1/2	terminated	no	—
NCT07645469	1	recruiting	no	Fred Hutch Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (sponsor-investigator)
- **Product info:**https://clinicaltrials.gov/study/NCT07645469
- **Notes:** Academic program with no commercial sponsor and no medical-information line.